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/ 2026 AHFTC Board Certification Question Bank

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/ Practice Exam - 1

Correct
Answer

Show

Practice Exam - 1

Question 80 of 117



A 50-year-old man comes to your office for routine follow-up. He has a history of dyslipidemia, coronary artery disease, and is status-post primary percutaneous intervention for left circumflex territory myocardial infarction two years ago. One year ago, he experienced stent thrombosis while taking high doses of non-steroidal anti-inflammatory drugs for gout. This resulted in a large circumflex territory infarct, was complicated by ventricular fibrillation arrest, and resulted in placement of a dual chamber implantable cardioverter-defibrillator. He feels generally well, though while walking into his home with a bag of groceries he gets short of breath.

Medications include atorvastatin 80 mg daily, aspirin 81 mg daily, metoprolol succinate 200 mg daily, sacubitril/valsartan 97/103 mg twice daily, spironolactone 25 mg daily, and ticagrelor 90 mg twice daily.

On examination, blood pressure is 112/72 mmHg, pulse is 60 beats per minute. Examination demonstrates no jugular venous distension, a PMI that is minimally displaced laterally from the mid-clavicular line, a soft I/VI systolic murmur, and no peripheral edema. The remainder of the examination is normal.

Echocardiography demonstrates a moderately dilated left ventricle, with an LV internal diameter in systole at 5.9 cm. Ejection fraction is calculated at 32%, with an akinetic inferior and inferolateral wall. There is trivial mitral regurgitation. Laboratory studies reveal a GFR of 65 ml/min, and a BNP of 65 pg/ml. Which of the following is the best next step?



Electrocardiogram



A. Start dapagliflozin 10 mg daily



B. Refer for upgrade to cardiac resynchronization therapy (CRT-D)

C. Start ivabradine 5 mg twice daily

D. Add lisinopril 10 mg daily

Commentary

References

Testing Point: The Board-certified AHFTC specialist should know that dapagliflozin is indicated for patients with heart failure with reduced ejection fraction, regardless of whether the patient has diabetes or not.

Answer: A. Start dapagliflozin 10 mg daily.

This patient has a reduced ejection fraction and continues to have at least NYHA class II symptoms. Therefore, dapagliflozin 10 mg daily is indicated, based on the results of the DAPA-HF trial. The key enrollment criteria were LVEF $\leq$ 40%, NYHA Class II-IV symptoms, which this patient meets.

Answer B: The QRS complex is narrow on electrocardiogram, obviating the utility of cardiac resynchronization therapy.

Answer C: The heart rate is 60, and atrial paced, making ivabradine less helpful, as this agent is indicated when the heart rate remains $\geq$ 70 beat per minute when a patient is on maximally tolerated doses of beta-blockers or have a contraindication to beta-blocker use.

Answer D: There is no role for addition of ACE-inhibitor to target dose sacubitril/valsartan, and in fact this combination is contraindicated because of the risk of angioedema.

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